INDUSTRY STARTER PACKS FOR ENTERPRISE ARCHITECTS

Healthcare

The complete set — what is in it, what each piece is for, and the order to pick them up

Two volumes, six spreadsheets, three decks and the worked example they were all built against. Read this first: it says which file answers which question, and which three you need in your first week.

South African lens. Sourced throughout. October 2026.

First edition · October 2026

Thotnr Consulting

Contents

THE SET

Guide 1 · What is in this set

GUIDE 1

What is in this set

Thirteen files. Three of them matter in your first week.

This set exists so that an architect who has never worked in healthcare can walk into a hospital group or a provincial health department and be useful. Not eventually — within a quarter.

It comes in four parts and a worked example. The two books teach and instruct, the spreadsheets are what you actually fill in, the decks are what you put on a screen in front of other people, and the Ravensmere files are a complete, populated example of what the finished thing looks like — defects and all.

Read these three first

> **IF YOU READ NOTHING ELSE**
>
> **`01 Learner guide`** — read it once, front to back, before you start. It is the industry.
>
> **`02 Workbook`** — never read front to back. Open it at the phase you are in and work the numbered steps.
>
> **`03 Excel templates / Healthcare_EA_Metamodel_TEMPLATE.xlsx`** — the empty repository. Everything the workbook tells you to do lands in it.

The whole set

Folder	File	What it is	When you use it
01	Healthcare — a starter pack for enterprise architects	The learner guide. 259 pages, 25 items, 114 figures, 865 sourced citations.	Read once, keep beside you
02	Healthcare — the architecture workbook	The ADM workbook. 237 pages, 143 numbered steps, 121 printable forms.	Every working day
03	Healthcare_EA_Metamodel_TEMPLATE.xlsx	The empty repository. 117 sheets, with change control, a computed index, 70 computed checks and the six sheets the example lacks.	From day one
03	Healthcare_Architecture_Comparison_Tool_TEMPLATE.xlsx	Baseline against target, with a reading on every difference against the record, the classification and the clock.	Phase E, then quarterly
03	Healthcare_EA_Workshop_Capture.xlsx	Live capture for the seven sessions that produce rows.	In the room
03	Healthcare_EA_Transcription_Forms.xlsx	57 cards matching the printed forms.	Typing up paper
03	Healthcare_EA_Principles_and_Standards.xlsx	32 principles and 24 standards, ready to adopt or argue with.	Preliminary phase
03	Healthcare_EA_Maturity_Assessment.xlsx	29 scored questions across 11 domains, plus a register of ten absences.	First month, then annually
04	Healthcare_EA_Executive_Briefing.pptx	31 slides for the executive committee.	End of the first quarter
04	Healthcare_EA_View_Templates.pptx	49 slides: 36 ArchiMate 4 views, drawn and guided.	Whenever you draw
04	Healthcare_EA_Workshop_Pack.pptx	55 slides facilitating the seven packs.	In the room
05	Healthcare_TOGAF_Metamodel.xlsx	The worked example — a populated repository for an invented provider.	When you want to see what finished looks like
05	Healthcare_ArchiMate4_Views.pptx	27 worked ArchiMate 4 views of that repository.	Alongside the view templates

The two books, and why they are separate

	Volume 1 — the learner guide	Volume 2 — the workbook
Read	once, front to back	never front to back
Answers	what this industry is and how it runs	what do I do on Monday
Shelf life	keep it as reference	consumed — you write in it
Opens at	chapter 1	the phase you are in

Six things in here that will surprise an architect from another industry

1. **Downtime is clinical.** Not a service-level matter. The planning assumption for a serious outage in this sector is weeks, not hours, and volume 1 names the incidents that establish it.
2. **The strongest business case is medico-legal, not clinical.** In an audit of 27 facilities, three of nine paid claims were conceded because nobody could find the medical record. Provinces pay around R1,5bn a year.
3. **Almost nothing that matters is specified nationally.** No national procedure classification, no national tariff since 2010, no national medicines dictionary, no present-on-admission flag, no national identity-merge policy, and no retention instrument at all. The architect's job is making those decisions locally and writing them down.
4. **The sector's own official numbers do not reconcile**, and the disagreements are large, recent and published by the same institutions. Volume 1 chapter 2 teaches reading them as a discipline.
5. **A measured South African failure of record linkage exists** and it is worse than anyone assumes: in 13,499 infant tests, 4% of infants matched exactly on all four demographic details.
6. **NHI is a contingency that validates the work, not the reason for it.** No section is in force, and everything an NHI architecture would require is already mandated by instruments in force today.

The six sheets this set adds

The supplied repository declares six industry types — patient, encounter, clinical protocol, care setting, medical device, consent — and holds not one instance of any of them. The workbook creates the six sheets that absence makes necessary, and the template ships them empty:

Sheet	What it holds	Created in
`067 Consent`	A consent with a scope, a grant date, an expiry and a withdrawal — not a flag	Item 5
`068 Medical Device`	Risk class, software version, calibration and maintenance dates, network segment	Item 7
`069 Code System`	The edition in use and the window it is valid for	Item 2
`070 Measure Definition`	Numerator, denominator, exclusions, boundary rule — so a derived figure travels with its rule	Item 5
`071 Disclosure Event`	Who was told what, under which authority, against which consent	Item 5
`072 Retention Clock`	The period, the event it runs from, the instrument and the destruction authority	Item 2

What the worked example teaches

`05 Worked example` is a populated repository whose defects are left in deliberately, because each one is a caricature of a documented real pattern. Volume 1 chapter 21 reads it end to end and volume 2 works against it throughout. The headline: **every relationship in it resolves and almost none of them means anything.** Nine relationship sheets were built by pairing two lists in row order — human resources owns medicines management, the billing system supports emergency care — and referential integrity is perfect. A repository can pass every integrity check and be worthless.

Alongside that: not one of the fourteen data entities is mastered in a system that could own it; the patient identifier is classified `Public` beneath an entity classified `Restricted`; seven work packages read `Delivered` before any has reached its start date; and the resilience tier label carries no information at all.

How this set handles what it does not know

The research behind volume 1 runs to 219,000 words across 869 sources and carries **217 items that could not be verified**. Where something could not be established, the books say so and name what would settle it, rather than estimating. Where two official sources disagree, both are printed and the disagreement is stated.

> **TWO THINGS TO RE-CHECK BEFORE YOU RELY ON THEM**
>
> **The Constitutional Court's NHI judgment** — heard 5 to 7 May 2026, still reserved at the end of October 2026.
>
> **The SNOMED CT National Release Centre for South Africa** — the country joined SNOMED International on or about 14 May 2026, and no release centre was operating at the time of writing. If one now is, volume 1 chapter 12 and the terminology steps in volume 2 change.

Three invented organisations

Driehoek Health Group (private: nine acute hospitals, two day clinics) and **Rietvlei District Health Service** (public: one district hospital, three community health centres, 22 fixed clinics) carry the teaching and the worked answers. **Ravensmere Health Network** is the populated repository in folder 05. All three are fictional and every figure attributed to any of them is illustrative.

Real organisations, vendors and regulators are named normally throughout, with sources and dates. Vendors are named as examples, never as recommendations. No text, table or figure is reproduced from ArchiMate, TOGAF, HL7, SNOMED CT, ICD, DICOM, IHE, openEHR, ISO, IEC or any other standard or licensed model.